

Introduction to Facilitated Positional Release

Michael J. Terzella, D.O.
Department of Osteopathic Manipulative Medicine
mterzell@nyit.edu

Session Objectives

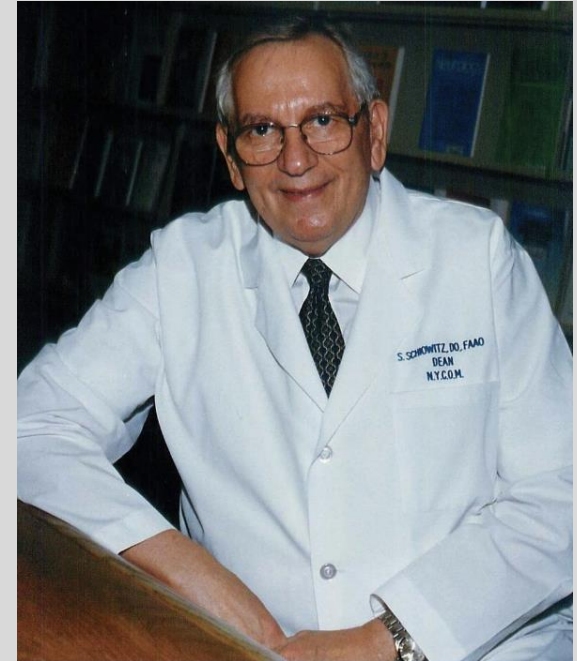
- Discuss the history of the FPR technique
- Explain the principles of the FPR technique
- Explain the physiologic mechanisms involved
- Discuss the indications and contraindications of the FPR technique
- Explain how to apply the principles of FPR to treat any given somatic dysfunction based on your knowledge of anatomy and the function of the given region.

History of FPR

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- Developed by:
Stanley Schiowitz, D.O., F.A.A.O.
 - Dean of NYCOM 1991-2002
- 90 seconds was too long for Dr. S.
- "IO"



Basic Principles of FPR

- Passive
- Indirect

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Basic Principles of FPR

- Muscle Spindle Reflex
 - Theory – Compressing muscles past their neutral position results in a “quieting” of gamma motor gain to the muscle spindle, leading to a resetting of the tension and hypertonicity of the muscle fibers.

Basic Principles of FPR

What are the basics of performing the technique?

- Neutralize the position of the dysfunction
- Add a facilitating force to the point being treated.
(Facilitating force = Compress, traction, or torque)
- Maintain the facilitating force and guide the dysfunction into its freedom & stay there for 3-5 seconds
- Release facilitating force and guide dysfunction back to baseline position and reassess

Indications

- Any condition where somatic dysfunction is involved
 - Note: Treat larger muscle groups first before treating joints
 - ie. In someone with neck pain secondary to whiplash, treat the cervical soft tissue first, then move on to the spinal segments.

Contraindications of FPR (1 of 3)

- Patient unable to tolerate treatment positions
 - Modify technique or use a different technique
- Radicular pain when performing to c-spine using compression (similar to Spurling Maneuver)
 - Reposition, try with traction, or try another technique

Contraindications of FPR (2 of 3)

- Any condition of the region being treated where introduction of torque, traction, or compression is contraindicated
 - Hip replacement (treatment position for herniated disc pain)
 - Shoulder pathology (treatment position for Rib 1)
 - Chronic shoulder dislocation/separation (any treatment that moves and /or puts force into the shoulder)

Contraindications of FPR (3 of 3)

- Recent trauma, fracture or dislocation at area involved with treatment
- Caution (but, not contraindication)
 - Osteoporosis
 - Malignancy
 - Rheumatologic Disorders
 - Congenital malformations
 - Stenosis
 - Other clinical diseases (use your clinical judgement)

Side Effects of FPR

➤ Transient:

- Fatigue
- Soreness
- Stiffness

Due primarily to the restoration of circulation to previously dysfunctional tissue and release of muscular waste products.

[Foundations of Osteopathic Medicine 4th edition, Chapter 40G: Facilitated Positional Release](#)

Case

A 29-year-old male with no significant past medical history presents to your office complaining of achy low back pain for 10 years. He was a gymnast in high school and has had pain intermittently since then. When his back “goes out” the severity of the pain is a sharp 8/10. On a normal day, it is 4/10 at the end of the day. Nothing makes it better or worse.

Case (continued)

The pain is non-radiating. He has no associated numbness, tingling, weakness, or bowel or bladder function changes. He brought in recent lumbosacral spine x-rays and an MRI which which showed some mild degenerative changes.

Case (continued)

PMHx: childhood asthma

PSx: no surgeries

Soc Hx: denies alcohol, drugs, tobacco use

Family History: both parents and 2 siblings alive and well

ROS: Only positive for the back pain

Case (continued)

Physical Exam:

Lower Extremity Motor 5/5 B/L

L4 and S1 reflexes 2/4 B/L

No gross sensory deficit in the lower extremities

Negative straight leg raise B/L

Negative costovertebral angle tenderness B/L

Case (continued)

Physical Exam (continued):

Negative FABER test

B/L PVMS

B/L Psoas spasm

B/L hamstring spasm

Right Quadratus lumborum spasm

L5 ERS right

Case (continued)

Assesment:

Low Back pain

Muscle spasm

Somatic Dysfunction – Lumbar

Somatic Dysfunction – Lower Extremity

Case (continued)

Plan:

Low Back pain – Hamstring, psoas, QL, Paraspinal mm. stretches. Stretch each region for 20 seconds, relax the stretch for a few seconds, restretch 20 seconds and relax 2 more times.

Muscle spasm – Stretches as above

Somatic Dysfunction – Lumbar

OMT – Soft tissue, MET, FPR

Somatic Dysfunction – Lower Extremity

OMT - MET

Case (continued)

How did we treat the L5 ERS right with FPR?

Neutralize

Compress (Facilitating force)

Freedoms

Reassess

Summary

FPR:

- Passive, indirect technique
- Physiologic mechanism - Muscle spindle
- Method - Neutralize, add facilitating force, guide to freedoms, return to baseline, reassess

Core References

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Medicine

DiGiovanna, Amen and Burns. An Osteopathic
Approach to Diagnosis and Treatment, 4th Ed

[Chapter 15: Facilitated Positional Release](#)

Foundations of Osteopathic Medicine, 4th Edition

[Foundations of Osteopathic Medicine 4th edition,](#)

[Chapter 40G: Facilitated Positional Release](#)

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Foundations of Osteopathic Medicine, 5th Ed

[Foundations of Osteopathic Medicine 5th edition,](#)

[Chapter 48: Facilitated Positional Release](#)

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